National Cancer Control Programme (NCCP) 00304 · clinician draft

Carboplatin and paclitaxel · lung cancer

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · carboplatin and paclitaxel

Active medicines: paclitaxel + carboplatin

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg orally on treatment day; 80 mg orally on each of the following two days	Each Day 1 chemotherapy visit
Ondansetron	16 mg orally	Each chemotherapy treatment day Review QT risk, interactions and individual dose choice.
Antiemetic dexamethasone reference	12 mg orally on treatment day; 8 mg orally daily for the next three days	Each Day 1 chemotherapy visit The team must reconcile paclitaxel allergy-prevention steroids with anti-sickness steroids and the aprepitant interaction into one prescription. Do not add courses or take leftover steroids yourself.
Metoclopramide	10 mg orally three times daily as prescribed	Subsequent days NCIS high-risk row does not define an end day or PRN qualifier; prescriber must specify duration, suitability and interaction risks.
Paclitaxel dexamethasone premedication	Source reference 20 mg oral or intravenous (IV)	Oral approximately 6 and 12 hours before paclitaxel, or IV 30 minutes before Reconcile with antiemetic steroid; source advises considering 12 mg on treatment day when aprepitant is added. Consultant may reduce/omit only under explicit guidance.
Chlorphenamine	10 mg IV	30 minutes before paclitaxel
Famotidine	20 mg IV	30 minutes before paclitaxel Omission requires consultant guidance. Source cautions particularly with every-three-week paclitaxel; monitor closely for hypersensitivity.

Additional phase-specific support

- Take allergy-prevention and anti-sickness medicines exactly as prescribed. The team combines steroid instructions into one plan; do not add leftover tablets or miss a premedication without telling the team.
- Ask for help with poor intake, vomiting, diarrhoea or reduced urine. Follow your individual fluid advice rather than a fixed target.
- Plan transport if medicines make you drowsy. Bring your medicine list, symptom record and treatment card.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00304 v8 · pNCCP 00304 pp 5–6; NCIS v 8 Table 1 · NCCP NCIS antiemetics V 8

Sources checked 2026-11-02 · SACTCheck v0.84.28 · Regimen-level drafts; prescriber and oncology-pharmacy verification required